THE WHITE ARMY

GIT CASE PRESENTATION

•Presented by:-
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-Final year MBBS

Patient Details

Name: Mr xyz
Age: 39 years
Sex: male
Address: Nandigram, West Bengal
Occupation: Labourer
Education: Class 10th
DOA: 1/09/2021
DOE: 3/09/2021

Chief Complaints

Abdominal distension X 6 months
Swelling of lower limbs X 5 months
Yellowish discolouration of eyes X 7 days
Fever X 3 days

History of present illness

Abdominal distension X 6 months

The patient was apparently alright 6 months back when he noticed abdominal distension.

- Insidious in onset
- Gradually progressed to the current state
- Diffuse in nature
- Not associated with abdominal pain or obstipation
- Not associated with dyspnea



. Image of abdominal distension which is diffuse in nature.

Swelling of lower limbs X 5 months

-Insidious in onset
-Initially involved only the feet but then gradually progressed uptil knees
-Present throughout the day and reduces on lying down or legs raised
-No associated scrotal swelling
-No history of chest pain or palpitations



Yellowish discolouration of eyes X 7 Days

- Noticed by his wife
- Not associated with itching, high coloured urine, or pale stools



Fever X 3 days

-High grade
-Associated with chills and sweating
- Continuous
-Relieved with over the counter anti pyretics
-Associated with myalgia
-No association with joint pain, rash.

-Patient also complaints of increased sleepiness during the day and sleeplessness at night X 5 days.
- No history of confusion or altered mental sensorium.

Past History

-Episode of jaundice 8 months back for which he took native treatment.
-He had a history of previous COVID-19 infection 1 month back.
-No history of TB, asthma, Diabetes mellitus, Hypertension, Heart disease and Seizures.
-No history of blood transfusions or tattooing.
-No history of recent travels.
-No history of any previous abdominal surgeries.

Family History

-No history of similar complaints in the family.

Personal History

OCCUPATION: Labourer
DIET: Mixed, decreased appetite since 1 week
SLEEP: Disturbed sleep at night and increase sleepiness during the day.
BOWEL & BLADDER: Normal
HABITS: -Smoker
- 150 ml of alcohol daily since last 20 years
- No history of i.v. drug abuse

SOCIAL: Happily married and in monogamous relationship with his wife.

SUMMARY

His condition probably involves the GI system, in particular the hepatobiliary system. My provisional diagnosis in this patient is decompensated chronic liver disease with symptoms suggestive of portal hypertension and hepatic encephalopathy with probable aetiologies being chronic alcohol consumption and other possible differentials include viral hepatitis and toxin induced liver damage.

General Physical Examination

Patient is awake, irritable and well oriented to time, place and person.
Patient is emaciated and malnourished.

Height: 150 cm
Weight: 58 kg

PALLOR: Present
ICTERUS: Present
EDEMA: Present - Pitting type uptil knees

No cyanosis, no clubbing, no lymphadenopathy

Examination

-Parotid swelling is present on both sides
-Palmar erythema is present
-3 spider nevi are present, two on the chest and one on the back.
- Gynaecomastia is present
- Flapping tremors are present.



Spider Nevi



Gynaecomastia



Palmar erythema



Flapping Tremors

VITALS

TEMPERATURE: 100.6 F, measured orally

PULSE: 74/min, regular rhythm, normal character and volume, no delays, all peripheral pulses are palpable and equally bilateral.

BLOOD PRESSURE: 100/64 mm of Hg measured in right upper limb in supine position.

RESPIRATORY RATE: 17/min abdominothoracic
JVP: Not elevated

Systemic Examination

Examination of Oral Cavity

LIPS: Normal with no evidence of cracks, fissures or scars
TEETH: Multiple caries
TONGUE: Normal
There are no ulcers or patches on the palate or mucosa

Inspection

-Abdomen is uniformly distended and flanks are full.
-Umbilicus appears to be displaced downwards and horizontally slit.
-All abdominal quadrants move equally with respiration.
-Skin appears shiny with no stria or scratches noted.
-No discolouration of skin is seen.
- Engorged veins present over the abdomen.
- No visible peristalsis or pulsations.
-No scars or sinuses.
-On head raising test, no divarication of recti seen.
- Hernial orifices appear normal.
-External genitalia appears normal.

Palpation

On superficial palpation:

-The abdomen is not warm and non tender.
-No abdominal rigidity or guarding.
-Abdominal girth is 105 cm. Spinoumbilical distance is 20 cm on both sides. Xiphoumbilical distance is 24 cm and distanve from umbilicus to pubic symphysis is 19 cm, suggesting displacement of umbilicus downwards.
- Direction of flow of blood in engorged veins is away from the umbilicus both above and below.

On Deep Palpation

-No palpable mass appreciated.

Percussion

Percussion of liver:

- Upper border of liver dullness is percussed at right 5th intercoastal space along the mid-clavicular line and the lower border is not palpable.

Percussion of Spleen:

Castell's method: Dullness is observed in the 9th ICS of the anterior axillary line.

- Traube’s space is dull to percussion
- Shifting dullness present.
- Fluid thrill was appreciated.

Auscultation

-Bowel sounds are present
-No venous bruit or hum.

Examination of other systems:

CNS EXAMINATION:

-Awake and irritable, well oriented to time, place and person.
-Plantar reflex is flexor.
-No focal neurological deficits present.
-Flapping tremors are present.
-No other involuntary movements observed.

Provisional Diagnosis

My diagnosis for this patient is decompensated liver disease probably cirrhosis of liver with features of pirtal hypertension and hepatic encephalopathy grade 2 due to chronic alcohol consumption.